15.1 Antenatal Care*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "This is my first pregnancy; I do not know what is normal." **Objective** • First pregnancy, 12 weeks • Cannot name the danger signs • First antenatal visit	**ACTUAL** Inadequate health knowledge related to first pregnancy as evidenced by questioning and inability to state danger signs.	• By the next visit, names at least five danger signs and the visit schedule.	• Explain the schedule of antenatal visits and the planned tests. • Teach the danger signs: vaginal bleeding, severe headache, blurred vision, swelling of face or hands, fewer fetal movements, leaking fluid, fever. • Teach nutrition and the use of folic acid and iron, as prescribed. • Advise no smoking, alcohol or non-prescribed drugs.	• Met when she states the danger signs and the schedule.
Subjective • "I feel sick every morning." **Objective** • Vomits once a day • Weight steady • Not dehydrated	**ACTUAL** Ineffective nausea self-management related to hormonal changes of early pregnancy as evidenced by morning nausea and vomiting.	• Less nausea and intake maintained.	• Suggest small, frequent dry meals and a few crackers before getting up. • Advise avoiding fatty and strong-smelling food. • Ask her to come in if she cannot keep fluids down.	• Achieved when nausea is manageable.
Subjective *No specific complaint* **Objective** • Hb 10 g/dL • Missed earlier visits	**RISK** Risk for impaired maternal-fetal dyad as evidenced by risk factors: anemia and inadequate prenatal care.	• Attends scheduled visits; hemoglobin improving.	• Give iron as prescribed and teach how to take it. • Set up visit reminders. • Check fetal growth at each visit.	• Outcome reached when she attends and Hb improves.

* **References for this care plan:** [35], [47], [95], [119]. Full entries are listed in the References sectiFull entries are listed in the References section of the CPMN book.